Vital Signs Monitoring Policy

SHIFA General Hospital (Fictional)

DEMO / SYNTHETIC HOSPITAL DATA — NOT FOR REAL CLINICAL USE

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1. Purpose

This policy sets how often vital signs are measured and how abnormal results must be escalated, so that a worsening patient is recognised early.

2. Parameters

Each set of vital signs includes temperature, pulse, blood pressure, respiratory rate, oxygen saturation (SpO2), level of consciousness (AVPU) and a pain score from 0 to 10.

3. Monitoring Frequency

- Stable ward patients: at least once every 8 hours (once per shift).
- Post-operative patients: every 15 minutes for the first hour, every 30 minutes for the next 2 hours, hourly for the next 4 hours, then every 4 hours if stable.
- Blood transfusion: before starting, 15 minutes after starting, then hourly and at the end of the transfusion.
- ICU and HDU: continuous monitoring with charting every hour.

4. Early Warning Score (EWS) and Response

- Score 0 to 4 (low risk): continue routine monitoring. If any single parameter scores 3, inform the nurse in-charge.
- Score 5 or 6 (medium risk): inform the responsible doctor within 30 minutes and increase monitoring to at least hourly.
- Score 7 or more (high risk): call the Rapid Response Team on extension 1777 immediately, start continuous monitoring and stay with the patient.

5. Pain Assessment

Pain is assessed with every set of vital signs on a scale of 0 to 10. A score of 4 or more must be reported to the doctor or treated with the prescribed analgesic, and reassessed within 30 minutes.

6. Documentation and Equipment

Readings are recorded immediately and no later than 30 minutes after measurement. Abnormal values are highlighted. Previous values must never be copied forward. Monitors and thermometers are calibrated every 6 months by Biomedical Engineering and faulty devices are tagged and removed.

7. Children

Children under 12 years are monitored with the Paediatric Early Warning Score (PEWS), which uses age-specific ranges. The adult EWS chart is not used for them.